PRACTICE EXAM 31 / EXAM SET 05

Domain 3: Clinical Patient Care

84 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Patient Intake and Vitals

> **SCENARIO** An older adult needs a routine weight. The available standing scale is rated to 150 kg, while a recent verified weight is 176 kg. Clinic policy requires equipment rated for the expected patient load and prohibits testing an overloaded scale.

What should the assistant arrange?

A. A trial on the 150 kg scale to see whether it still gives a number.

B. A measurement while the patient shifts some weight onto a nearby chair.

C. An approved scale with sufficient load capacity before weighing.

D. The recent weight entered as a measurement obtained today.

Q02 | Patient Intake and Vitals

> **SCENARIO** During a routine repeat blood pressure measurement, the assistant notices that a manual aneroid gauge points to 12 mm Hg while fully depressurized. Clinic policy requires a zero reading before use and prohibits staff from applying numerical corrections.

What should the assistant do?

A. Subtract 12 mm Hg from each measured endpoint.

B. Inflate the cuff once to reset the pointer automatically.

C. Record the reading without mentioning the gauge finding.

D. Remove the gauge from service and use a verified replacement.

Q03 | Patient Intake and Vitals

SCENARIO A school-age child's blood pressure is entered into a clinic screening tool. The tool requires age, sex, and measured height percentile to select its comparison table, but the height field is blank.

What completes the screening workflow?

A. Obtain the required height information before using the tool's comparison result.
B. Select the adult reference table to avoid delaying intake.
C. Use the child's weight percentile in the height field.
D. Select the tallest-height column for every child.

Q04 | Patient Intake and Vitals

SCENARIO Two trained staff count simultaneously for 60 seconds. The apical count is 88 beats and the radial count is 74 beats. The clinic asks for the numerical apical-radial difference and directs clinician review of a difference.

What difference should be reported?

A. 7 beats/min
B. 74 beats/min
C. 14 beats/min
D. 162 beats/min

Q05 | Patient Intake and Vitals

SCENARIO A parent brings a 3-week-old infant for a scheduled weight check and reports a home rectal temperature of 38.2 degrees C. The clinic protocol requires immediate clinician evaluation of a reported rectal temperature of at least 38 degrees C in an infant younger than 28 days.

How should the assistant handle the report?

A. Continue routine rooming and mention the temperature after the visit.
B. Notify the clinician immediately and keep the infant available for prompt evaluation.
C. Advise the parent to start a fever medicine without review.
D. Disregard the report because it was measured at home.

Q06 | Patient Intake and Vitals

SCENARIO A patient says, "The biggest reason I came is the new ankle swelling, but I also need a work form." The assistant is choosing the chief complaint heading before gathering the full history.

Which entry best reflects the patient's stated visit priority?

A. Work-form completion only.
B. Confirmed heart failure.
C. New ankle swelling.
D. Routine visit with no symptoms.

Q07 | Patient Intake and Vitals

> **SCENARIO** A patient taking two differently named over-the-counter cold products brings both packages. Each lists the same active ingredient. The clinician has asked the assistant to identify overlapping ingredients during medication reconciliation.

What should the assistant document and communicate?

A. Only the product with the larger package.
B. That the products are different medicines because their brands differ.
C. Both product names and the shared active ingredient for clinician review.
D. A new reduced dosing schedule chosen by the assistant.

Q08 | Patient Intake and Vitals

> **SCENARIO** A patient with stable breathing has a pulse oximeter displaying an interference warning under a bright examination lamp. Its instructions state that intense external light can interfere and direct use of its opaque probe cover before repeating the reading.

Which action follows the device instruction?

A. Enter the warning message as a confirmed oxygen saturation.
B. Increase the displayed saturation manually by five points.
C. Place two sensors on the same finger simultaneously.
D. Apply the approved opaque cover and repeat the measurement.

Q09 | Patient Intake and Vitals

> **SCENARIO** A resting respiratory rate of 18 breaths/min is measured while the patient receives prescribed oxygen at 2 L/min by nasal cannula. The intake screen has fields for respiratory rate and current respiratory support.

Which documentation is complete?

A. Respiratory rate 18 breaths/min on room air.
B. Respiratory rate 20 breaths/min after adding the oxygen flow.
C. Respiratory rate 18 breaths/min; oxygen 2 L/min by nasal cannula.
D. Oxygen 2 L/min, with the measured respiratory rate omitted.

Q10 | Patient Intake and Vitals

> **SCENARIO** An infant's head circumference is being followed over time. The measuring tape has a visibly stretched section. Clinic equipment policy requires an intact, nonstretch tape and a repeat measurement if tape integrity is uncertain.

What is the appropriate measurement action?

A. Use an intact approved tape and obtain a new measurement.
B. Pull the damaged tape tighter to compensate.
C. Add a standard 1 cm correction to the result.
D. Use the previous visit's value as today's measurement.

Q11 | Patient Intake and Vitals

> **SCENARIO** An older adult says, "Penicillin made me feel sick to my stomach years ago." The EHR already lists penicillin allergy, but there are no reaction details. Clinic procedure requires documenting the reported reaction and routing uncertain allergy classifications for clinician review.

What should the assistant do?

A. Record the reported stomach symptoms and request review of the existing allergy entry.
B. Delete the allergy because stomach symptoms cannot require review.
C. Change the reaction to anaphylaxis to make the alert more visible.
D. Leave the reaction blank because the event was years ago.

Q12 | Patient Intake and Vitals

> **SCENARIO** A home-monitor screenshot lists a glucose value of 7.2 but does not show units. The clinic EHR requires mg/dL, and no device model or unit setting has yet been confirmed.

How should the assistant handle the value?

A. Enter 7.2 mg/dL because that is the EHR default.
B. Enter 720 mg/dL by moving the decimal.
C. Verify the original units before entering a converted clinical value.
D. Record 7.2 as a clinic-performed laboratory result.

Q13 | Patient Intake and Vitals

> **SCENARIO** A screening tool asks whether a symptom occurred during the past 14 days. A patient describes a single episode three months ago and explicitly denies episodes during the past 14 days.

How should the assistant complete the time-limited screening item?

A. Mark the current-period item positive because the symptom ever occurred.
B. Record the response for the past 14 days and document the older episode separately.
C. Change the tool's interval to three months.
D. Omit the item because past symptoms make the question unanswerable.

Q14 | Patient Intake and Vitals

> **SCENARIO** A preprocedure form asks whether pregnancy is possible. The patient says, "I do not know." The clinic protocol sends uncertain responses to the clinician before the planned procedure.

Which entry and action are appropriate?

A. Document the uncertain response and notify the clinician under the protocol.
B. Select No because the patient did not say Yes.
C. Select Yes and document pregnancy as confirmed.
D. Remove the question from the form.

Q15 | General Patient Care

> **SCENARIO** An authorized oral prescription is 18 mg/kg/day divided into three equal doses. The verified weight is 20 kg, and the suspension contains 120 mg per 5 mL. The order is complete and has been clinically verified.

What volume is required for each dose?

A. 15 mL
B. 7.5 mL
C. 1.5 mL
D. 5 mL

Q16 | General Patient Care

> **SCENARIO** A verified medication reference supplied with an order permits 10-15 mg/kg per dose for this patient. The patient weighs 24 kg, but the prescribed single dose is 400 mg. Clinic policy requires clarification of doses outside the supplied range before administration.

What should the assistant do?

A. Hold the dose and request clarification because the permitted range is 240-360 mg per dose.
B. Administer 400 mg because it is below 24 x 20 mg.
C. Reduce the dose independently to 360 mg and administer it.
D. Treat 400 mg as a daily dose and split it without an order.

Q17 | General Patient Care

Which instrument is used to grasp skin edges during a minor procedure?

A. The toothed tissue forceps.
B. The needle holder.
C. The scissors.
D. The hemostat.

Q18 | General Patient Care

SCENARIO An orally disintegrating tablet is ordered. Its package instruction requires dry hands and peeling back the foil because pushing the tablet through can damage it.

Which preparation follows the instruction?

A. Push the tablet through the foil using a damp thumb.
B. With dry hands, peel the foil and remove the intact tablet.
C. Crush the tablet into the unopened foil pocket.
D. Soak the sealed blister in water before opening it.

Q19 | General Patient Care

SCENARIO After an authorized eye medication, the provider-approved instruction calls for gentle pressure at the inner corner of the closed eye for one minute. The assistant is reinforcing that instruction.

What should the patient demonstrate?

A. Firm pressure directly over the center of the eyeball.
B. Gentle pressure near the inner corner without pressing on the eyeball.
C. Repeated forceful blinking for one minute.
D. Rubbing the closed eyelids with a tissue.

Q20 | General Patient Care

SCENARIO A minor-procedure team uses a marked neutral zone for sharps transfer. The provider asks for a scalpel while both hands are occupied. The team's protocol requires placing it in the neutral zone and announcing placement.

How should the assistant transfer it?

A. Insert the scalpel handle into the provider's occupied hand.
B. Hold the blade toward the provider until a hand becomes free.
C. Place the scalpel in the neutral zone and announce that it is available.
D. Toss the capped scalpel onto the drape near the incision.

Q21 | General Patient Care

SCENARIO During a procedure, a sterile-gloved assistant needs an additional sterile dressing from an unsterile supply drawer. A circulating staff member is available, and the clinic workflow assigns drawer access and outer-wrapper handling to that person.

How should the sterile-gloved assistant obtain the dressing?

A. Open the unsterile drawer directly with the sterile gloves.
B. Reach into the drawer and consider the gloves sterile if they look clean.
C. Ask the circulating staff member to retrieve and present it through the sterile-supply workflow.
D. Leave the field to search the drawer without changing gloves afterward.

Q22 | General Patient Care

> **SCENARIO** During ordered ear irrigation, the device's supplied instruction sets a maximum pressure setting of 2. The assembled unit is on setting 4. The patient is comfortable, and irrigation has not begun.

What should the authorized assistant do before starting?

A. Start on setting 4 and reduce it only if pain develops.
B. Set the device within the specified permitted range.
C. Keep setting 4 but shorten the procedure.
D. Remove the pressure control so flow is unrestricted.

Q23 | General Patient Care

> **SCENARIO** An older adult needs assistance standing after an abdominal procedure. The transfer plan prohibits placing a gait belt across the incision and specifies an approved support harness with two trained staff. Both staff and the harness are available.

Which method follows the plan?

A. Fasten a gait belt tightly across the incision.
B. Lift the patient under both arms without the harness.
C. Use the approved harness with the two trained staff.
D. Ask a visitor to pull the patient up by the wrists.

Q24 | General Patient Care

> **SCENARIO** A patient is practicing with an ordered walker under a clinician-approved mobility plan. The patient repeatedly pushes it so far ahead that reaching the handles requires bending forward. The plan instructs keeping the walker close enough for an upright supported stance.

Which cue reinforces the plan?

A. Push it farther ahead so you take the longest possible step.
B. Keep the walker close enough that you can stand upright while holding its handles.
C. Lean your trunk forward until your shoulders pass the handles.
D. Lift the walker above waist level between steps.

Q25 | General Patient Care

> **SCENARIO** A provider-approved home-care handout says patients with reduced skin sensation need clinician review before using a heating pad. An older adult reports numbness over the proposed treatment area and plans to sleep on the pad.

What should the assistant reinforce?

A. Sleep on the pad if its cover feels soft.
B. Use the highest setting for a shorter period without review.
C. Test the temperature only with the numb area.
D. Obtain clinician guidance before applying heat to the numb area.

Q26 | General Patient Care

> **SCENARIO** An AED announces a pad-contact problem. The patient is unresponsive, emergency response and trained resuscitation are underway, and dense chest hair prevents one pad from adhering. The AED kit instruction directs rapid clipping of the pad area with the supplied clipper and replacement with a new pad.

What should the trained assistant do as part of the team's response?

A. Place the loose pad over clothing to hold it in place.
B. Follow the supplied clipping and new-pad instruction promptly.
C. Keep pressing Analyze without correcting pad contact.
D. Apply conductive gel over the hair beneath the loose pad.

Q27 | General Patient Care

> **SCENARIO** At the start of a scheduled video visit, the assistant sees that the patient is actively driving. The clinic's telehealth procedure requires the patient to be safely parked before the clinical encounter continues.

What should the assistant do?

A. Pause the encounter and have the patient resume only after parking safely.
B. Begin the assessment while asking the patient to watch the screen.
C. Ask the patient to hold the phone closer while driving.
D. Record that the patient is stationary to avoid rescheduling.

Q28 | General Patient Care

> **SCENARIO** An EHR medication field is configured for dose in mg. The authorized administration record says 30 mg given using 1.5 mL. The assistant is about to enter 1.5 in the mg field.

Which entry belongs in that dose field?

A. 1.5 mg, because that was the measured syringe volume.

B. 31.5 mg, combining the two recorded numbers.

C. 20 mg, calculated by dividing dose by volume.

D. 30 mg, with 1.5 mL recorded in the appropriate volume field.

Q29 | General Patient Care

> **SCENARIO** A medication task appears as Scheduled. The patient says another staff member already administered it, but the administration record is incomplete. The clinic's duplicate-dose prevention procedure requires verification with that staff member before another dose is prepared.

What should the assistant do?

A. Verify whether the dose was administered before preparing another dose.

B. Give it again because Scheduled means it was not given.

C. Mark it Not given without checking the report.

D. Ask the patient to choose whether a second dose is needed.

Q30 | General Patient Care

> **SCENARIO** Two medication orders are being independently checked under a clinic protocol. The first check recorded a concentration of 10 mg/mL, but the bottle presented for administration reads 20 mg/mL. The second check is not yet complete.

What should the assistant do?

A. Resolve the concentration discrepancy before administration.

B. Sign the second check because the first checker already signed.

C. Use the previously calculated volume without reviewing the bottle.

D. Cover the bottle label so both records show the same concentration.

Q31 | General Patient Care

> **SCENARIO** An authorized non-IV injectable product's instructions specify no massage after injection. The injection is complete, and the patient asks the assistant to rub the area vigorously.

Which response follows the product instruction?

A. Massage it firmly because every injection needs dispersal.
B. Massage only if the patient supplies a clean tissue.
C. Change the documented route to permit massage.
D. Explain that the area should not be massaged under this product's instructions.

Q32 | General Patient Care

> **SCENARIO** A trained assistant is reinforcing an ordered ostomy pouch care plan. It directs emptying the drainable pouch at one-third full. The patient plans to wait until it is completely full to reduce emptying frequency.

Which instruction matches the plan?

A. Wait until it is completely full before emptying.
B. Empty the pouch when it is about one-third full.
C. Puncture the pouch if emptying becomes inconvenient.
D. Stop using the pouch overnight.

Q33 | General Patient Care

> **SCENARIO** During authorized nebulizer preparation, the device's assembly diagram shows the medication cup upright. The cup is tilted almost horizontal although all prescribed parts and solution are correct.

What setup correction is needed?

A. Position the medication cup upright as shown.
B. Add extra medication to compensate for the tilt.
C. Increase the prescribed dose frequency.
D. Remove the cup and pour solution into the tubing.

Q34 | General Patient Care

> **SCENARIO** After a minor procedure, the signed plan permits showering after 24 hours but prohibits soaking the site for seven days. The patient says, "So I can take a bath tomorrow."

Which clarification correctly reinforces the plan?

A. A bath and a shower have the same restriction under this plan.
B. All washing must stop for seven days.
C. A shower after 24 hours is allowed, but soaking remains prohibited for seven days.
D. The assistant can shorten the soaking restriction if the dressing is dry.

Q35 | General Patient Care

Which description defines the supine position?

A. Lie on your back with your face upward.
B. Lie face down on your abdomen.
C. Lie on your left side with your upper knee flexed.
D. Sit upright with your feet on the floor.

Q36 | General Patient Care

> **SCENARIO** A provider requests sterile gauze for blotting near a procedure site. The assistant finds two visually identical packages: one marked Sterile and intact, the other marked Nonsterile. Neither package has been opened.

Which supply matches the request?

A. The intact package labeled Sterile.
B. The Nonsterile package because it is unopened.
C. Either package after touching the gauze with sterile gloves.
D. The Nonsterile package after wiping its outer wrapper.

Q37 | General Patient Care

> **SCENARIO** An authorized oral medication is available as a tablet and a liquid, but the signed order specifically names the tablet formulation. The patient requests the liquid instead, and the clinic requires prescriber or pharmacist approval for formulation substitution.

What should the assistant do?

A. Give the same numerical amount in mL as the tablet dose in mg.
B. Obtain the required approval before substituting the liquid.
C. Choose the liquid because both products share a drug name.
D. Document a tablet dose while administering liquid.

Q38 | General Patient Care

> **SCENARIO** A patient using a provider-approved elastic support asks whether the support should cover a newly placed bulky dressing. The written plan says to leave the dressing uncompressed and obtain clinician review if the support overlaps it.

What should the assistant reinforce?

A. Keep the dressing uncompressed and obtain the specified review before overlap.
B. Compress the dressing tightly to make the support fit.
C. Remove the dressing without an order so the support lies flat.
D. Cut a window in the support without asking the clinician.

Q39 | General Patient Care

> **SCENARIO** An authorized injection task requires a patient-specific product that arrived in a cracked glass vial. The outer carton and printed expiration date are acceptable, and no dose has been withdrawn.

Which preparation decision is appropriate?

A. Withdraw from the vial because the expiration date has not passed.
B. Set the damaged vial aside and obtain an intact authorized replacement.
C. Pour the medication into a clean specimen cup.
D. Tape the crack and store the vial for the next patient.

Q40 | General Patient Care

> **SCENARIO** A clinical procedure order requires documenting the patient's response 15 minutes after completion. It finishes at 10:42 a.m. The assistant is arranging the required observation without changing the order.

At what time is the specified reassessment due?

A. 10:42 a.m.
B. 10:47 a.m.
C. 10:57 a.m.
D. 11:12 a.m.

Q41 | General Patient Care

> **SCENARIO** A child preparing for a routine examination wants a parent nearby. The clinician approves the parent's presence, and clinic policy allows the child to choose which arm is examined first when either is clinically suitable.

Which approach supports participation?

A. Promise that nothing in the visit could ever hurt.
B. Keep the approved parent nearby and offer the permitted choice of starting arm.
C. Require the parent to leave despite the clinician's approval.
D. Ask the child to decide which clinical procedures are necessary.

Q42 | General Patient Care

> **SCENARIO** A clinician demonstrates a hand exercise and provides a written plan of five repetitions twice daily. During teach-back, the patient correctly demonstrates the movement but says it should be performed five times every hour.

Which part of the instruction needs correction?

A. The movement, because correct demonstration proves nothing.
B. The repetition count: it should automatically be doubled.
C. The written plan, which the assistant may replace with hourly exercise.
D. The frequency: the written plan is twice daily.

Q43 | Infection Control and Safety

> **SCENARIO** After a splash-risk task, goggles have visible blood spots on the front. The clinic's removal procedure directs handling them only by the rear strap, followed by the specified reprocessing and hand hygiene.

How should the assistant remove them?

A. Lift them off using the rear strap without touching the contaminated front.
B. Grip both front lenses with bare fingertips.
C. Slide the contaminated lenses across the face.
D. Place the goggles directly into a clean supply drawer.

Q44 | Infection Control and Safety

> **SCENARIO** The posted handwashing procedure requires at least 20 seconds of scrubbing before rinsing. A staff member wets the hands, applies soap, scrubs for five seconds, and reaches for the rinse.

Which step requires correction?

A. Rinse immediately because soap contact alone is sufficient.
B. Dry the soapy hands without rinsing.
C. Replace the rinse step with donning gloves.
D. Continue scrubbing until the required duration is met.

Q45 | Infection Control and Safety

> **SCENARIO** A high-touch workstation surface is covered with blood droplets. The available disinfectant is labeled only for hard, nonporous surfaces, but this workstation has an absorbent fabric cover. Clinic policy sends contaminated porous furnishings to its designated environmental service process.

What should the assistant do?

A. Apply the hard-surface product and assume its label covers fabric.
B. Spray more product until the fabric is soaked.
C. Cover the wet fabric with paper and return it to use.
D. Restrict use and arrange the designated porous-surface process.

Q46 | Infection Control and Safety

> **SCENARIO** A reprocessing detergent's instructions require fresh solution for each load. After the first load, the solution looks clear and a second basket is ready.

What should the assistant prepare?

A. The same solution because visible clarity proves it is reusable.
B. A fresh solution for the second load.
C. The same solution with an unmeasured splash of concentrate.
D. A mixture of used solution and a different cleaner.

Q47 | Infection Control and Safety

> **SCENARIO** The cleaning guide for a reusable instrument lumen requires a brush that contacts its full inner circumference. The brush on the tray is visibly narrower than the lumen. An approved correctly sized brush is available.

Which action meets the cleaning instruction?

A. Use the approved brush that contacts the full inner circumference.

B. Pass the narrow brush more quickly and skip the sizing requirement.

C. Proceed to sterilization without brushing because heat removes debris.

D. Block the lumen opening with gauze and clean only its exterior.

Q48 | Infection Control and Safety

> **SCENARIO** A dynamic-air-removal steam sterilizer's required air-removal test shows an unacceptable uneven pattern. The clinic instruction says not to run instrument loads after a failed test until the designated service review clears the unit.

What should the assistant do?

A. Run a larger load to force the remaining air out.

B. Withhold instrument loads and arrange the required service review.

C. Ignore the result because the chamber becomes hot.

D. Use an instrument load as the next air-removal test pack.

Q49 | Infection Control and Safety

> **SCENARIO** A centrifuge stops after a tube-breakage alarm. The clinic's sealed-bucket spill procedure instructs leaving the lid closed, isolating the unit, and contacting trained personnel before opening it. The assistant has not been trained for this spill procedure.

What should the assistant do?

A. Open the lid immediately to count the fragments.

B. Reach into the bucket wearing ordinary examination gloves.

C. Keep the lid closed, isolate the unit, and contact the trained response staff.

D. Restart the centrifuge to settle the broken glass.

Q50 | Infection Control and Safety

> **SCENARIO** After an injection, the needle's safety feature will not engage. The sharp remains intact and is being controlled away from others. Clinic procedure requires immediate disposal into the nearby approved sharps container without recapping or repair.

What should the assistant do?

A. Force the mechanism with the free hand beside the needle tip.
B. Recap the needle so it can be carried to maintenance.
C. Leave the needle on the counter for someone to inspect.
D. Dispose of it directly in the sharps container using the stated safe-handling procedure.

Q51 | Infection Control and Safety

> **SCENARIO** A fire alarm is sounding. A stable patient is seated in an examination room and requires guidance to the clinic's clear evacuation route. The emergency plan assigns this assistant to escort that patient while another trained team handles the fire.

Which action follows the assignment?

A. Leave the patient to search for a fire extinguisher.
B. Wait for smoke to enter the room before moving.
C. Send the patient alone to retrieve personal belongings first.
D. Escort the patient along the designated clear route.

Q52 | Infection Control and Safety

> **SCENARIO** A power failure leaves a powered examination chair reclined. The patient is comfortable but cannot stand from that position. The device manual permits a specific manual-return lever operated by trained staff; a trained colleague is available.

What should the assistant arrange?

A. Pulling hard on the motorized backrest until the gears move.
B. Having the patient slide off the reclined chair alone.
C. Connecting the chair to an exposed improvised electrical lead.
D. The trained colleague's use of the approved manual-return method with patient support.

Q53 | Infection Control and Safety

SCENARIO A box of clinical supplies exceeds the clinic's one-person handling limit. The workplace instruction requires using an available cart and obtaining help to load it; the route is clear.

What handling method follows the instruction?

A. Carry the box alone because the route is short.
B. Balance it on one shoulder to keep one hand free.
C. Obtain loading assistance and use the cart.
D. Drag it through a patient-care area by its torn flap.

Q54 | Infection Control and Safety

SCENARIO A staff member's sleeve becomes visibly soaked with blood during care. The clinic exposure-control plan requires removing contaminated clothing promptly into the designated container and checking for skin or mucous-membrane contact under the exposure procedure.

Which response follows the plan?

A. Continue wearing it until the end of the shift.
B. Put a clean gown over the soaked sleeve and skip the check.
C. Take the clothing home in a lunch bag for washing.
D. Remove the contaminated clothing as directed and check for exposure requiring evaluation.

Q55 | Infection Control and Safety

SCENARIO A cleaning concentrate has a splash warning, and its supplied instruction requires adding concentrate slowly to a measured volume of water. The water is ready; the assistant is about to pour water into an empty container of concentrate instead.

Which correction follows the instruction?

A. Add the concentrate slowly to the measured water as directed.
B. Pour the water rapidly into the concentrate to shorten contact time.
C. Shake the concentrate while its cap is open.
D. Combine the liquids in any sequence because the final amounts match.

Q56 | Infection Control and Safety

> **SCENARIO** A protective face shield used for chemical handling is visibly cracked across the viewing area. Its manufacturer instructs replacing cracked shields. A compatible intact shield is available.

What should the assistant select?

A. The intact compatible replacement shield.
B. The cracked shield with tape across the viewing area.
C. No shield if the task takes less than a minute.
D. The cracked shield beneath ordinary glasses.

Q57 | Infection Control and Safety

> **SCENARIO** During room turnover, the approved checklist requires cleaning a reusable keyboard cover after every patient. The cover looks clean after a visit in which no procedure was performed.

Which action completes turnover?

A. Skip it because no soil is visible.
B. Clean and process the cover according to the between-patient checklist.
C. Clean only the keys touched with visibly dirty gloves.
D. Wait until the end of the week to avoid wear.

Q58 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A specified CLIA-waived assay requires both low and high controls to pass before testing. The low control is acceptable, but the high control is above its assigned range. The run has not been approved.

What is the correct testing status?

A. Patient testing remains on hold because both required controls have not passed.
B. Testing may proceed because one control is acceptable.
C. Only high patient results must be withheld.
D. Average the two controls and compare that average with the low range.

Q59 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A waived urine test's instructions require reading at five minutes and prohibit interpretation after ten minutes. A forgotten device is first inspected at twenty minutes and shows a faint test line.

How should the assistant handle it?

A. Report a positive result from the faint late line.
B. Report a negative result because the line is faint.
C. Treat it as unread within the valid window and repeat under the test instructions.
D. Record the read time as five minutes so the result can be used.

Q60 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A urine collection instruction requires a clean, dry container free of cleaning-agent residue. A patient brings urine in a household jar that still smells of detergent. An approved recollection container is available.

What should the assistant arrange?

A. Dilution with water to reduce the detergent concentration.
B. Transfer to a new cup and label the specimen acceptable.
C. Report the odor as proof of a urinary infection.
D. Recollection in the approved container.

Q61 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A stool transport kit requires adding specimen only to a marked fill line to preserve its stated specimen-to-medium ratio. A patient returns a container filled to the top, well above the line. The laboratory instruction requires recollection when overfilled.

What should the assistant do?

A. Pour off fluid until the container reaches the line.
B. Arrange recollection under the kit instructions.
C. Add extra transport medium from an unrelated kit.
D. Send it without noting the overfill.

Q62 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** During an ordered spirometry effort, a patient becomes dizzy and asks to rest. The clinic testing protocol requires stopping the maneuver, seating the patient safely, and notifying the clinician before deciding about additional attempts.

Which response follows the protocol?

A. Demand completion because stopping invalidates the attempt.
B. Stop, keep the patient safely seated, and obtain clinician review.
C. Have the patient stand and walk while continuing the blow.
D. Record a completed acceptable effort despite the interruption.

Q63 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A near-vision card is designed for a 40 cm test distance, as printed on the card. A patient holds it 20 cm away. The supplied screen procedure requires the designated distance.

What should the assistant correct?

A. Score the result as standard because the letters are readable.

B. Double the reported acuity denominator to compensate.

C. Position the card at 40 cm before scoring the screen.

D. Replace near-vision screening with a distance chart without an order.

Q64 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A mechanical peak-flow meter retains its previous maximum until its cursor is manually returned to the starting position. Its instructions require resetting before each blow. The patient is ready for the second attempt, but the cursor still shows the first result.

Which preparation is needed?

A. Keep the cursor at the first result so the readings can be added.

B. Move the cursor to the top of the scale.

C. Return the cursor to the specified starting position.

D. Record the first reading again without another blow.

Q65 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** Before a waived urine strip test, an assistant touches all reagent pads with a bare finger. The test instruction says to avoid touching pads and to replace a strip if this occurs.

What should the assistant use?

A. The touched strip after wiping the pads dry.

B. The touched strip with only the last pad interpreted.

C. The touched strip after rinsing it with water.

D. A new strip handled according to the instructions.

Q66 | Point of Care Testing and Laboratory Procedures

SCENARIO For an authorized skin test, the provider supplies a numbered placement map matching each test site to a specific allergen and control. The assistant's worksheet lacks the site numbers even though observations have been measured.

What must be resolved before reporting site-specific observations?

A. Assign allergen names by the relative size of each reaction.
B. List observations from largest to smallest and omit site identity.
C. Treat every observed reaction as the positive control.
D. Match and record each observation with the verified placement-map site.

Q67 | Phlebotomy

SCENARIO The first evacuated tube has filled during a routine venous draw, and a second ordered tube is ready. The collection procedure directs stabilizing the needle holder while the free hand removes and replaces tubes without moving the inserted needle.

Which technique follows the procedure?

A. Advance the needle farther each time a fresh tube is attached.
B. Stabilize the holder and exchange the tubes with the free hand.
C. Let go of the holder so both hands can pull off the first tube.
D. Withdraw and reinsert the same needle between tubes.

Q68 | Phlebotomy

SCENARIO A citrate coagulation collection is ordered. A verified laboratory alert lists hematocrit 58% and directs staff to consult the laboratory before collection above 55%, because the laboratory must determine whether an adjusted collection tube is needed.

What should the assistant do?

A. Remove an estimated amount of citrate independently.
B. Consult the laboratory before collecting into the routine tube.
C. Ignore the alert if the routine tube reaches its fill line.
D. Switch to an unrelated additive without consulting the laboratory.

Q69 | Phlebotomy

> **SCENARIO** A processing instruction requires 1,800 x g. The verified centrifuge chart for the installed rotor lists 3,000 rpm = 1,200 x g and 3,700 rpm = 1,800 x g. The display accepts rpm.

Which setting matches the required processing force?

A. 1,800 rpm
B. 3,000 rpm
C. 3,700 rpm
D. 1,200 rpm

Q70 | Phlebotomy

> **SCENARIO** Before a scheduled venous assay, the patient reports taking a high-dose biotin supplement that morning. The laboratory's preparation instruction flags recent high-dose biotin for clinician/laboratory review before collection. It does not authorize the assistant to stop the supplement or reschedule independently.

What should the assistant do?

A. Tell the patient to stop the supplement permanently.
B. Proceed without mentioning it because supplements are not medicines.
C. Cancel all the patient's tests independently.
D. Document the supplement and obtain the required preparation review.

Q71 | Phlebotomy

> **SCENARIO** A timed blood collection is scheduled for 10:00 a.m. The specimen is actually obtained at 10:18 a.m. Clinic procedure requires the actual time and notification of deviations from the ordered collection schedule.

What should be recorded and communicated?

A. 10:00 a.m., because that was the ordered time.
B. The appointment arrival time instead of the collection time.
C. A blank time so the laboratory can infer the schedule.
D. The actual 10:18 a.m. collection time and the timing deviation.

Q72 | Phlebotomy

> **SCENARIO** After routine venipuncture, the needle is safely discarded and gauze is in place. The clinic aftercare instruction requires direct pressure with the arm extended. The patient starts bending the elbow tightly over the gauze.

Which instruction follows the aftercare plan?

A. Bend the elbow as tightly as possible instead of applying pressure.
B. Rub the site briskly to stop bleeding.
C. Keep the arm extended and apply direct pressure over the site.
D. Remove the gauze immediately to let the site air-dry.

Q73 | Phlebotomy

> **SCENARIO** An ordered fasting collection's written preparation says plain water is permitted. A stable patient has avoided all fluids overnight because of a belief that fasting forbids water, and asks whether a sip is allowed.

Which clarification follows this order's instructions?

A. Plain water is permitted under the supplied preparation instructions.

B. Every fasting order prohibits all water.

C. Water is allowed only if it contains sugar.

D. The assistant should change the fasting test to a nonfasting test.

Q74 | Phlebotomy

> **SCENARIO** A patient withdraws consent for an ordered venous draw before needle insertion and says, "Stop; I need to speak with the clinician." The assistant has not punctured the skin.

What should the assistant do?

A. Stop the preparation, make the equipment safe, and notify the clinician of the request.

B. Proceed because the laboratory order is signed.

C. Ask another staff member to hold the arm down.

D. Document a completed collection without obtaining blood.

Q75 | Phlebotomy

> **SCENARIO** A collected venous tube is leaking around its closure, and the outer transport material is wet. The laboratory instruction prohibits shipping leaking primary containers and requires specimen-suitability review through its designated process.

How should the assistant handle it?

A. Place it in a larger bag and ship it as acceptable.

B. Contain it safely and obtain the required review before transport.

C. Pour the specimen into any available unlabeled tube.

D. Wipe only the outside and conceal the leak from the laboratory.

Q76 | Phlebotomy

> **SCENARIO** A capillary collection protocol permits a controlled warming pack at 40 degrees C for the stated interval. The available adjustable pack is set at 50 degrees C, above its approved skin-contact limit. Collection has not begun.

What should the assistant do?

A. Use the pack only after establishing the approved temperature and procedure.

B. Apply it at 50 degrees C for the full interval.

C. Wrap it in extra gauze and assume the temperature is acceptable.

D. Skip the temperature check because warming is optional.

Q77 | Phlebotomy

SCENARIO A capillary assay requires exactly 0.02 mL collected in its approved fixed-volume device. The kit defines 1 mL as 1,000 microliters. Compatible devices are labeled 20, 50, 100, and 200 microliters.

Which device matches the collection volume?

A. 200 microliters
B. 20 microliters
C. 50 microliters
D. 100 microliters

Q78 | Phlebotomy

SCENARIO A capillary sample collector's instruction says to keep it horizontal so blood advances by capillary action without an air break. The assistant is holding it upright, and a gap has formed. The instruction requires a new collector if a gap occurs.

What should the assistant use for the repeat collection?

A. A new collector held in the specified horizontal position.
B. The same collector after shaking the blood across the gap.
C. The same collector after drawing blood through it by mouth.
D. A collector lubricated inside with tap water.

Q79 | EKG and Cardiovascular Testing

Where is standard V5 placed relative to an already correctly positioned V4?

A. At the left midaxillary line, horizontally level with V4.
B. At the right sternal border in the fourth intercostal space.
C. At the left anterior axillary line, horizontally level with V4.
D. At the left midclavicular line in the second intercostal space.

Q80 | EKG and Cardiovascular Testing

SCENARIO A stable patient's tracing has intermittent poor-contact warnings despite correctly connected cables and prepared skin. The electrode package is past its use-by date, and its gel is visibly dry. The approved troubleshooting guide directs replacing such electrodes.

Which correction addresses the identified cause?

A. Increase filtering until the poor-contact warning is hidden.
B. Apply the expired electrodes more tightly with cable tension.
C. Replace the electrodes with fresh approved ones and reacquire the tracing.
D. Document the artifact as a confirmed rhythm abnormality.

Q81 | EKG and Cardiovascular Testing

> **SCENARIO** An ambulatory recorder's instructions say to press its event button once when a brief symptom occurs and record the time and activity in the diary. The patient asks what to do during a mild, brief familiar flutter without emergency-warning symptoms.

Which instruction matches the device plan?

A. Press it continuously for the rest of the day.
B. Press the event button once and record the symptom time and activity.
C. Remove the recorder whenever a symptom begins.
D. Record the symptom only at the end of the week from memory.

Q82 | EKG and Cardiovascular Testing

> **SCENARIO** A patient scheduled for an exercise EKG asks whether to skip the morning heart medication. The written test preparation says medication withholding requires an individualized clinician instruction, but the chart contains none.

What should the assistant do?

A. Tell every exercise-test patient to skip all heart medicines.
B. Tell the patient to double the medication to prepare for exercise.
C. Choose an instruction based on how the patient feels that morning.
D. Obtain clinician-specific medication instructions before advising the patient.

Q83 | EKG and Cardiovascular Testing

> **SCENARIO** An ordered continuous EKG segment must contain ten seconds. The device paper speed is 50 mm/second, but a paper-feed problem stops this segment after 400 mm. The patient is stable, and the setup guide requires a complete segment before submission.

How should the assistant handle this recording?

A. Label it as ten seconds because that was the selected program.
B. Recognize that it contains eight seconds and obtain the required complete segment.
C. Report it as twenty seconds by applying a standard speed from memory.
D. Stretch the printed strip until it measures the expected length.

Q84 | EKG and Cardiovascular Testing

> **SCENARIO** A reusable ambulatory recorder displays Upload failed after return. The device instruction says not to erase stored data until successful transfer is confirmed. The recording remains in its memory, and technical support is available.

What should the assistant do?

A. Erase memory to make room before checking whether transfer succeeded.

B. Mark the study successfully uploaded because recording time elapsed.

C. Assign the recorder to another patient without resolving the failed transfer.

D. Preserve the recording and follow the approved upload troubleshooting process.

ANSWER KEY & EXPLANATIONS

Practice Exam 31 | Domain 3: Clinical Patient Care | 84 scored questions

01 C	02 D	03 A	04 C	05 B	06 C
07 C	08 D	09 C	10 A	11 A	12 C
13 B	14 A	15 D	16 A	17 A	18 B
19 B	20 C	21 C	22 B	23 C	24 B
25 D	26 B	27 A	28 D	29 A	30 A
31 D	32 B	33 A	34 C	35 A	36 A
37 B	38 A	39 B	40 C	41 B	42 D
43 A	44 D	45 D	46 B	47 A	48 B
49 C	50 D	51 D	52 D	53 C	54 D
55 A	56 A	57 B	58 A	59 C	60 D
61 B	62 B	63 C	64 C	65 D	66 D
67 B	68 B	69 C	70 D	71 D	72 C
73 A	74 A	75 B	76 A	77 B	78 A
79 C	80 C	81 B	82 D	83 B	84 D

DETAILED EXPLANATIONS

Q01 | Correct answer: C

An approved scale with sufficient load capacity before weighing.

The expected load exceeds the scale's rated capacity. The assistant obtains suitable equipment rather than overloading it, partially supporting body mass, or presenting an old value as current.

Q02 | Correct answer: D

Remove the gauge from service and use a verified replacement.

The device fails the stated readiness check. An improvised subtraction does not establish that the gauge is accurate throughout its operating range.

Q03 | Correct answer: A

Obtain the required height information before using the tool's comparison result.

The supplied tool cannot choose its intended reference without the required height information. Weight percentile or an adult table is not an interchangeable input.

Q04 | Correct answer: C

14 beats/min

For the simultaneous one-minute counts, 88 - 74 = 14 beats/min. Adding the counts or halving their difference does not describe the measured difference.

Q05 | Correct answer: B

Notify the clinician immediately and keep the infant available for prompt evaluation.

The reported age and temperature meet the supplied immediate-review criterion. Home source attribution should be preserved without dismissing clinically important information or initiating treatment independently.

Q06 | Correct answer: C

New ankle swelling.

The patient explicitly identifies the new swelling as the main reason for the visit. The assistant records the symptom rather than assigning a diagnosis or replacing the stated concern with paperwork.

Q07 | Correct answer: C

Both product names and the shared active ingredient for clinician review.

Listing both products and their shared ingredient preserves information needed to assess duplicate exposure. Brand differences do not establish different active ingredients, and dose changes require clinician direction.

Q08 | Correct answer: D

Apply the approved opaque cover and repeat the measurement.

The specified cover addresses the documented light-interference condition. A warning is not a validated saturation, and numerical adjustment cannot correct acquisition interference.

Q09 | Correct answer: C

Respiratory rate 18 breaths/min; oxygen 2 L/min by nasal cannula.

The record should retain the observed rate and the support present during measurement. Oxygen flow is not added to a respiratory rate, and the patient was not breathing room air alone.

Q10 | Correct answer: A

Use an intact approved tape and obtain a new measurement.

A stretched tape can distort the measurement. A new measurement with intact equipment is needed; neither an arbitrary correction nor a historical value supplies a valid current measurement.

Q11 | Correct answer: A

Record the reported stomach symptoms and request review of the existing allergy entry.

The assistant should preserve the patient's actual description and request authorized classification review. Unilaterally deleting the entry or inventing a severe reaction would reduce record accuracy.

Q12 | Correct answer: C

Verify the original units before entering a converted clinical value.

The numerical value cannot be safely converted without knowing its original units. The record must also preserve that the reading came from the patient's home device.

Q13 | Correct answer: B

Record the response for the past 14 days and document the older episode separately.

The response should match the tool's defined interval. The older event can be retained elsewhere without changing the meaning of the time-limited response.

Q14 | Correct answer: A

Document the uncertain response and notify the clinician under the protocol.

An uncertain screening response is neither a confirmed negative nor a diagnosis. Documenting it accurately allows the clinician to decide whether further evaluation is needed.

Q15 | Correct answer: D

5 mL

The daily dose is 18 mg/kg/day x 20 kg = 360 mg/day. Dividing by three gives 120 mg per dose; 120 mg x 5 mL / 120 mg = 5 mL per dose.

Q16 | Correct answer: A

Hold the dose and request clarification because the permitted range is 240-360 mg per dose.

The supplied range is 24 x 10 = 240 mg through 24 x 15 = 360 mg per dose. The 400 mg order exceeds it; the assistant obtains clarification rather than choosing a replacement prescription.

Q17 | Correct answer: A

The toothed tissue forceps.

The requested forceps are designed to grasp tissue and match the provider's named instrument. The other instruments serve needle handling, cutting, or clamping functions.

Q18 | Correct answer: B

With dry hands, peel the foil and remove the intact tablet.

The specified opening method protects this fragile dosage form. A usual push-through blister technique is not appropriate when the manufacturer specifically directs peeling.

Q19 | Correct answer: B

Gentle pressure near the inner corner without pressing on the eyeball.

The instruction describes gentle inner-corner pressure, not pressure on the globe. Rubbing or forceful blinking does not reproduce the prescribed technique.

Q20 | Correct answer: C

Place the scalpel in the neutral zone and announce that it is available.

The neutral-zone process avoids an unplanned hand-to-hand sharp transfer. Announcing placement helps the provider locate the instrument without reaching blindly.

Q21 | Correct answer: C

Ask the circulating staff member to retrieve and present it through the sterile-supply workflow.

The circulating role provides the supply without requiring sterile gloves to touch the unsterile drawer. Glove appearance does not establish that contact with an unsterile surface is safe.

Q22 | Correct answer: B

Set the device within the specified permitted range.

The unit must meet its operating limit before use. Patient comfort before irrigation does not authorize a pressure outside the supplied instruction.

Q23 | Correct answer: C

Use the approved harness with the two trained staff.

The specified harness avoids pressure across the incision while providing the planned level of assistance. Familiar transfer equipment is not appropriate when the plan explicitly prohibits its placement.

Q24 | Correct answer: B

Keep the walker close enough that you can stand upright while holding its handles.

The cue addresses the observed excessive reach and the plan's upright-stance goal. Extending the walker farther would increase the same positioning problem.

Q25 | Correct answer: D

Obtain clinician guidance before applying heat to the numb area.

Reduced sensation can limit recognition of excessive heat, and this report triggers the handout's review instruction. The assistant should not invent an alternative heat dose.

Q26 | Correct answer: B

Follow the supplied clipping and new-pad instruction promptly.

The supplied method corrects the identified adhesion problem so the AED can function. Clothing or an unsecured pad does not provide the required skin contact.

Q27 | Correct answer: A

Pause the encounter and have the patient resume only after parking safely.

The patient's current activity does not meet the clinic's safe participation requirement. The encounter can resume after the stated condition is met without falsifying the record.

Q28 | Correct answer: D

30 mg, with 1.5 mL recorded in the appropriate volume field.

Dose mass and administered volume are different quantities. The entry should match the field's units and retain volume separately where the system provides for it.

Q29 | Correct answer: A

Verify whether the dose was administered before preparing another dose.

A task label alone does not resolve a credible report of earlier administration. Verification addresses the duplicate-dose risk while preserving accurate documentation.

Q30 | Correct answer: A

Resolve the concentration discrepancy before administration.

The presented product does not match the concentration used in the first check. Independent checking requires resolving that difference, not repeating an earlier unchecked conclusion.

Q31 | Correct answer: D

Explain that the area should not be massaged under this product's instructions.

The supplied product instruction governs this aftercare step. Patient preference does not authorize a technique the instruction prohibits.

Q32 | Correct answer: B

Empty the pouch when it is about one-third full.

The clinician's supplied fill limit is one-third, and waiting until full does not follow it. Reinforcement should preserve the actual care plan rather than invent an alternative.

Q33 | Correct answer: A

Position the medication cup upright as shown.

The assembly problem is cup orientation, so the correction is mechanical and follows the device instructions. It does not justify changing the medication order.

Q34 | Correct answer: C

A shower after 24 hours is allowed, but soaking remains prohibited for seven days.

The plan distinguishes brief showering from soaking. The patient's statement needs correction without extending or shortening the provider's actual instructions.

Q35 | Correct answer: A

Lie on your back with your face upward.

Supine describes lying on the back facing upward. The other options describe different positions and would not implement the order.

Q36 | Correct answer: A

The intact package labeled Sterile.

Unopened packaging does not turn a nonsterile product into a sterile one. The product's designation must match the provider's requested supply.

Q37 | Correct answer: B

Obtain the required approval before substituting the liquid.

The request involves a formulation substitution that the stated workflow requires an authorized professional to approve. Equal numerical values in mg and mL do not establish an equivalent dose.

Q38 | Correct answer: A

Keep the dressing uncompressed and obtain the specified review before overlap.

The plan explicitly protects the dressing from compression and specifies how to resolve overlap. The assistant should not redesign the prescribed dressing or support.

Q39 | Correct answer: B

Set the damaged vial aside and obtain an intact authorized replacement.

A cracked primary container does not provide reliable product containment and integrity. An acceptable date on the carton does not correct the physical defect.

Q40 | Correct answer: C

10:57 a.m.

Adding 15 minutes to 10:42 a.m. gives 10:57 a.m. The interval starts from procedure completion, not from the earlier scheduled appointment.

Q41 | Correct answer: B

Keep the approved parent nearby and offer the permitted choice of starting arm.

A safe, bounded choice and an approved support person can help the child participate. The child is not responsible for selecting necessary care, and an absolute painless promise may be inaccurate.

Q42 | Correct answer: D

The frequency: the written plan is twice daily.

Teach-back reveals a frequency error despite correct technique. The assistant should reinforce the two daily sessions in the signed plan without changing the prescribed regimen.

Q43 | Correct answer: A

Lift them off using the rear strap without touching the contaminated front.

The rear strap provides the handling point specified by the removal procedure. Touching the contaminated front can transfer material to the hands or face.

Q44 | Correct answer: D

Continue scrubbing until the required duration is met.

The supplied procedure includes a minimum scrubbing duration that has not been completed. Gloves do not replace the required handwashing process.

Q45 | Correct answer: D

Restrict use and arrange the designated porous-surface process.

The product's labeled surface type does not include this fabric. The clinic's separate porous-material process addresses the mismatch without inventing a valid disinfection method.

Q46 | Correct answer: B

A fresh solution for the second load.

The stated reuse limit is one load, independent of appearance. Adding concentrate or another chemical does not validate reuse.

Q47 | Correct answer: A

Use the approved brush that contacts the full inner circumference.

Mechanical cleaning must reach the surfaces specified by the device guide. A subsequent process does not excuse omitting the required internal cleaning contact.

Q48 | Correct answer: B

Withhold instrument loads and arrange the required service review.

The test has failed the explicit readiness criterion for this unit. Chamber warmth alone does not verify the required air-removal performance.

Q49 | Correct answer: C

Keep the lid closed, isolate the unit, and contact the trained response staff.

The supplied response controls access to a potentially contaminated internal spill. The assistant follows the defined containment and escalation steps rather than opening an unfamiliar hazard.

Q50 | Correct answer: D

Dispose of it directly in the sharps container using the stated safe-handling procedure.

A failed safety feature does not authorize manipulating or recapping an exposed used needle. The stated direct-disposal process removes the sharp from further handling.

Q51 | Correct answer: D

Escort the patient along the designated clear route.

The assistant's assigned response is patient evacuation through the safe route. Leaving that assignment for equipment or belongings delays the planned patient protection.

Q52 | Correct answer: D

The trained colleague's use of the approved manual-return method with patient support.

The manual provides an approved recovery method that a trained staff member can use. Forcing the mechanism or improvising electrical power creates an unnecessary equipment and patient hazard.

Q53 | Correct answer: C

Obtain loading assistance and use the cart.

The load exceeds the stated individual handling limit. The available cart and loading assistance implement the specified control rather than relying on distance or improvisation.

Q54 | Correct answer: D

Remove the contaminated clothing as directed and check for exposure requiring evaluation.

The plan addresses both contaminated apparel and possible underlying exposure. Covering the sleeve does not complete either required step.

Q55 | Correct answer: A

Add the concentrate slowly to the measured water as directed.

The required mixing sequence is part of the product's handling instruction. Matching final amounts does not justify reversing a specified splash-control method.

Q56 | Correct answer: A

The intact compatible replacement shield.

The manufacturer identifies cracking as a replacement condition. Tape or a short task duration does not establish that the damaged protective barrier performs as intended.

Q57 | Correct answer: B

Clean and process the cover according to the between-patient checklist.

The clinic's between-patient requirement applies even without visible soil or a procedure. Appearance alone does not establish completion of the specified environmental process.

Q58 | Correct answer: A

Patient testing remains on hold because both required controls have not passed.

Acceptance requires both stated control conditions. One passing level cannot cancel the failure at the other, and control levels are not combined into an average.

Q59 | Correct answer: C

Treat it as unread within the valid window and repeat under the test instructions.

The first observation occurred outside the permitted interpretation window. The late appearance cannot establish a valid result for the earlier window, and the time must not be falsified.

Q60 | Correct answer: D

Recollection in the approved container.

Transferring the sample cannot remove material already introduced by the original container. The specimen does not meet the stated collection condition and requires the approved recollection process.

Q61 | Correct answer: B

Arrange recollection under the kit instructions.

The required ratio was not followed, and the supplied instruction specifies recollection. Pouring off material does not reconstruct the original validated preparation.

Q62 | Correct answer: B

Stop, keep the patient safely seated, and obtain clinician review.

The protocol's symptom-based stop condition has been met. Completing a technically acceptable tracing does not take priority over the stated patient-safety response.

Q63 | Correct answer: C

Position the card at 40 cm before scoring the screen.

The instrument's designated distance is part of the screen method. An unvalidated numerical correction does not turn an off-distance performance into a standard result.

Q64 | Correct answer: C

Return the cursor to the specified starting position.

The device cannot reliably display a new lower effort if the previous marker is left in place. Resetting prepares it for an independent second measurement.

Q65 | Correct answer: D

A new strip handled according to the instructions.

The strip meets the manufacturer's supplied replacement condition. Wiping or rinsing reagent pads does not restore their validated condition.

Q66 | Correct answer: D

Match and record each observation with the verified placement-map site.

Site-specific observations require the verified source map. Reaction size cannot identify which allergen or control was placed at a site.

Q67 | Correct answer: B

Stabilize the holder and exchange the tubes with the free hand.

The stated technique protects the established needle position during the tube exchange. Advancing, releasing control, or reinserting the used needle is not part of that procedure.

Q68 | Correct answer: B

Consult the laboratory before collecting into the routine tube.

The documented value meets the supplied consultation criterion. Adjusting additive amounts is not an independent assistant decision, and routine filling does not resolve the alert.

Q69 | Correct answer: C

3,700 rpm

The supplied rotor-specific chart maps the required force to 3,700 rpm. The numerical value in x g cannot simply be entered into a field labeled rpm.

Q70 | Correct answer: D

Document the supplement and obtain the required preparation review.

The report triggers the assay's explicit review condition. The assistant communicates the timing and product information without independently changing treatment or the testing plan.

Q71 | Correct answer: D

The actual 10:18 a.m. collection time and the timing deviation.

The specimen's timing record must describe what occurred. Recording the scheduled time would conceal information needed to evaluate the timed result.

Q72 | Correct answer: C

Keep the arm extended and apply direct pressure over the site.

The supplied aftercare method specifies direct pressure and an extended arm. Tight elbow flexion is not a substitute for the required pressure technique.

Q73 | Correct answer: A

Plain water is permitted under the supplied preparation instructions.

The order-specific preparation expressly allows plain water. The assistant can reinforce that instruction without changing the test or adding other permitted drinks.

Q74 | Correct answer: A

Stop the preparation, make the equipment safe, and notify the clinician of the request.

The procedure should not continue after the patient asks to stop. Making equipment safe and routing the request allows further discussion without forcing or falsely completing collection.

Q75 | Correct answer: B

Contain it safely and obtain the required review before transport.

The primary container fails the stated transport condition. Secondary containment protects handling but does not itself make the leaking specimen suitable for shipment or testing.

Q76 | Correct answer: A

Use the pack only after establishing the approved temperature and procedure.

The warming equipment must meet its supplied limit before skin contact. An unverified barrier or optional use does not validate an excessive setting.

Q77 | Correct answer: B

20 microliters

The specified conversion is 0.02 mL x 1,000 microliters/mL = 20 microliters. A 200-microliter device would collect ten times the required volume.

Q78 | Correct answer: A

A new collector held in the specified horizontal position.

The supplied instruction requires both correct orientation and replacement after an air gap. Shaking or adding other material does not reproduce the validated collection method.

Q79 | Correct answer: C

At the left anterior axillary line, horizontally level with V4.

Standard V5 is at the left anterior axillary line at the horizontal level of V4. The left midaxillary location at that level corresponds to V6.

Q80 | Correct answer: C

Replace the electrodes with fresh approved ones and reacquire the tracing.

The documented electrode condition matches the guide's replacement criterion. Hiding the warning does not restore the electrical contact needed for acquisition.

Q81 | Correct answer: B

Press the event button once and record the symptom time and activity.

The event marker and contemporaneous diary help associate the reported symptom with the recording. This device action does not diagnose the symptom or replace separate emergency instructions.

Q82 | Correct answer: D

Obtain clinician-specific medication instructions before advising the patient.

The supplied preparation does not authorize a general medication-hold rule. The assistant obtains individualized direction rather than altering the prescription.

Q83 | Correct answer: B

Recognize that it contains eight seconds and obtain the required complete segment.

Duration is distance divided by speed: 400 mm / 50 mm/second = 8 seconds. The selected program does not establish that the interrupted recording actually captured the ordered duration.

Q84 | Correct answer: D

Preserve the recording and follow the approved upload troubleshooting process.

The available recording must be retained until the required transfer confirmation occurs. Erasing it first could remove the only copy of the study.

End of Practice Exam